Path to early cancer care · Vantaan ja Keravan hyvinvointialue

Vantaa and Kerava: what to do next

Recommended actions

Ordered by where the pathway breaks

Action 1 of 3 1 2 3

Give patients a named GP (omalääkäri) they keep over time

First step
Pick the 2-3 health stations with the worst 7-day access and pilot named-GP lists there for patients aged 50+.

Owner: Wellbeing services county, primary care director

Step: Getting to a GP

25% lower odds of dying from any cause

Norway: patients with the same GP over 15 years vs 1 year

Source checked PubMed Central

Evidence details Find more evidence live

Show 11 other evidence-backed options

Where the delay sits (2025)

The delay sits at getting to a GP, which is worse than Finland.

Finland's value Gap to Finland's level

1 Getting to a GP

85.4% 30.7 points worse than Finland

Non-urgent GP visits waiting over 7 days
(lower is better)

0% Finland 54.7% 100%

Worse than Finland's level Sotkanet 6411

2 GP recognises warning signs

No number

Not measured in Finland. England publishes it by area: 17% of cancers first found through an emergency visit.

No public data

NHS England, Fingertips 2024/25

The four referral routes a county needs

3 GP to specialist referral

No number

HUS (the Helsinki University Hospital group) value: 74.1%

Referrals processed within 3 days HUS-wide value, not a county figure.
(higher is better)

0% Finland 76.8% 100%

No county figure Sotkanet 13

What if: Getting to a GP

If Vantaa and Kerava matched Finland's level: **30.7** fewer of every 100 non-urgent GP visits would wait over 7 days

Finland 54.7%

Vantaa and Kerava 85.4%

Arithmetic on the published shares, not a forecast of any single action. Sotkanet 6411 · 2025

The four referral routes a county needs

Does your county have a written route for each? Decision support, not diagnosis.

1 Fast track

For Bleeding Lump or swelling Trouble swallowing

GP does The GP checks the guideline for the exact criteria and, if they fit or the GP is unsure, refers or orders the test now rather than booking a routine follow-up.

Evidence (2)

2 Vague-symptoms work-up

For Vague symptoms

GP does The GP runs basic tests and, if nothing explains the symptoms, refers to a unit that investigates vague but serious symptoms (the Danish diagnostic-centre and UK Rapid Diagnostic Centre model).

Evidence (3)

3 GP check within two weeks

For Change that won't heal

GP does The GP checks the guideline criteria for this symptom. If they fit, refer or test now.

Evidence (2)

4 Safety-net and recheck

For Concerns that fit no other route

GP does If referral criteria aren't met, the GP books a review date and tells the person exactly when to come back.

Evidence (2)

Finland has no national referral criteria yet: Finnish cancer strategy roadmap 2026–2031, measure 2.5.2 Kansallinen syöpästrategia

Ask the evidence →

← Back to Vantaa and Kerava

Sources and limitations →